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Name: نادية ابراهيم سلطان محمد National ID: 25707021700348	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

النادية
ابراهيم سلطان محمد علام
٢٠
ثمن عبدالرحمن احمد الطراي
القوم - الجيزة

٢٥٧٠٧٠٢١٧٠٠٣٤٨

IX5472865

٢٠٢١/٠٦ ٢٥٧٠٧٠٢١٧٠٠٣٤٨

بطون عمل

متروجة

سنفلة

محمّد حلمي محمد مصطفى

٢٠٢٨/٠٦/٠٧ حتى

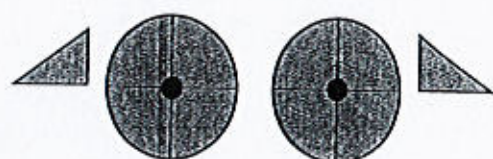
البطاقة سارية حتى

MDT FORM

١١ / 8 / 20 ٢٠٢٤ G

Name	فادى احمد السيد	MRN	4517 / 2024
Age	66	ID	CONV.CIV.24N
Menopausal status	Post-Menopausal	Phone	01155470004
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	irrelevant.	REASON OF MDT	post-op

Clinical Data

BX:	I.D.C			R	Ps1	L
ER:	PR:	HER 2:	Ki67:			
3/8	5/8	3				
Sonomammography:						
Mets Workup:				TNM		
put hist review HPI in Neoadj study				only review TC & S		

Recommendations: 4PT is 4pms

For adjuvant Herceptin +
tamoxifen Hormonal

MDT OUTCOME & PLAN

Adi Herceptin & XIA + Pore
TAM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: <u>نادية ابراهيم سلطان محمد</u>		
MR. : <u>533</u>		
Detailed Medical Report		
		السيد
تحية طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient is a 67-year-old with a history of post-menopausal breast cancer, clinical stage T4b N0 M0, estrogen-receptor negative, progesterone-receptor negative, HER2 3+ disease. After breast-conserving surgery and radiation therapy completed on 4/6/2025, the patient was started on adjuvant chemotherapy with Taxol and Carboplatin, followed by HER2-targeted therapy with Herceptin and endocrine therapy with Arimidex. Follow-up on 22/01/2026 confirmed the same pathology (T4b N0 M0, ER-ve, PR-ve, HER2 3+). The patient continued on the planned regimen of Taxol and Carboplatin for 18 weeks, with ongoing HER2-directed therapy (Herceptin) and was scheduled for a new echocardiogram. On 12/02/2026 the patient's left ventricular ejection fraction (EF) was 58 %. The plan was to administer cycle 7 of Herceptin subcutaneous (sc) and Arimidex; approval for tamoxifen was requested in the multidisciplinary team but was refused. Subsequent cycles 8 and 9 of Herceptin sc were scheduled, with pharmacy dispensing dates of 5/3/2026 and 26/3/2026. On 05/03/2026 the pharmacy note reiterated an EF of 58 % and preparation for cycle 8 of Herceptin sc and Arimidex, with a request for a repeat echocardiogram prior to the next cycle. On 26/03/2026 the pharmacy documented preparation for cycle 9 of Herceptin 600 mg sc together with Anastrozole and supportive care. The patient reported bone pain; zoledronic acid (Zometa) was continued despite dental issues. On 16/04/2026 an echocardiogram showed an EF of 69 % on 12/4/2026. The patient proceeded with cycle 9 of Herceptin 600 mg sc and Arimidex, with a new symptom assessment. On 10/05/2026 the patient received cycle 10 of Herceptin sc and Arimidex; examination noted right-upper limb edema, and physiotherapy was recommended. On 30/06/2026 the patient was prepared for cycle 12 of Herceptin sc and Arimidex; the scan was BIRADS 2 and a dexamethasone (DEXA) study was pending. On 26/07/2026 the plan was to repeat echocardiography; the scan remained BIRADS 2, dexamethasone pending, and the patient was scheduled for cycle 13 of Herceptin sc and Arimidex. On 16/08/2026 the EF was 65 % on 14/8. The patient was set for cycle 14 of Herceptin sc and Arimidex; a segmental hypoechoic lesion measuring 7 × 6		
تحريراً في ,		
التاريخ :	19-09-2026	الوقت :
الطبيب المعالج :	dina makram	
يعتمد ,		

mm was noted on triphasic CT.

On 06/09/2026 the patient was scheduled for cycle 15 of Herceptin sc and Arimidex. Ultrasound in July 2026 showed a suspicious hepatic focal lesion (HFL); CT of the chest, abdomen, and pelvis in August 2026 was negative. Cycles 16 and 17 were requested for triphasic imaging. To date, the patient has completed 15 cycles of Herceptin subcutaneous and Arimidex.



MDT form

A-Patient ID:

1- Name : نادية ابراهيم سلطان محمد علام

Sex:

3- Medical number:

Female

83251022

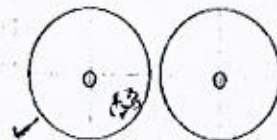
Old Med :

Old Med :

4517-2024

2-ID number:

Patient Age : 68 Y



Family history

Family History
+ve

Phone

Phone
01155470004

BMI

BMI

REASON OF MDT

REASON OF MDT
REMDT

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP
B

Date of MDT

Date of MDT

23/01/2025

Co morbidity

Co Morbidity
dm, htn
hypothyroidism

Menopausal
status

Menopausal Status
NOT ONETUP HIGH

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

rt 4 o'clock 3.7x2.7

5 o'clock 2x2 cm invading skin

Mets Workup :

free

BX
IDC

ER
9/8

PR
3/8

Her 2 Neu
3

SISH

Ki67

BX COMMENT

BX COMMENT

BX COMMENT

BX COMMENT

SISH COMMENT

T
CT4B
LOCALIZED

N
0

M
0

Sonomamniography

PATIENT HISTORY

cesm: rt breast multifocal malignant process with pathological enhancement BIRADS 6
for clip insertion (bracketing)

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for NACT TCHPX6 FOLLOWED BY RT BCS

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. علاء حبيب	RADIOLOGY	RADIOLOGY د. رجب البكري
SURGERY	SURGERY د. ايمن مرسى	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذ / نادية ابراهيم سلطان	Request No.	: 13009888
Patient No.	: 25707021700348	Request Date	: 04/06/2025
Path No.	: 1999/25	Verification Date	: 19/06/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 66 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data : Right breast cancer post NACT

Nature of the specimen : Right BCS & Right SLN biopsy .

Gross Examination :

Rt BCS specimen measured 9 x7x3 cm , covering skin ellipse measured 7x2 cm .

- Ill- defined clipped & wired greyish white firm tumour bed site measured 1 x1x1 cm , just beneath the skin

SLN biopsy : A fibrofatty tissue measured 4x3 cm enclosing 4 SLNs

• Frozen section: Free margins& Free SLNs (0/4)

Block designation:

- 1.medial
- 2.lateral
- 3.superior
- 4.inferior
- 5.deep
- 6.SLNS

7.Clipped area

3.Rest of cut section

Grossing by : Dr.Heba Hassan Adly

Microscopic examination:

- Histological type: No residual tumor
- Angioinvasion: No
- Lymph node involvement: Free SLNs (0/4) with no therapy effect
- Resection Margins: Free margins

Verified by :

Dr.Heba Hasan



Patient Name	: الأستاذ / نادية ابراهيم سلطان	Request No.	: 13009888
Patient No.	: 25707021700348	Request Date	: 04/06/2025
Path No.	: 1999/25	Verification Date	: 19/06/2025
Sex	: Female	Financial Category	: جومئاف روسي - مبادره
Age	: 66 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Histologic response to therapy: Pathologic complete response

Diagnosis:

Right Breast Conservative Surgery & Right SLN biopsy :

No viable tumor in a previously diagnosed invasive duct carcinoma

- Free margins
- Free SLNs (0/7)(No therapy effect)(confirmed by CK)
- Pathologic complete response
- Stage : ypTis , ypN0

Signature

Prof. Dr. Maha Guimei
Dr.Heba Hassan Adly

Verified by :

Dr.Heba Hasan



PATIENT NAME: NADYA IBRAHIM SULTAN MOHAMMED

GENDER: Female

DATE : 20-May-2025 12:39 PM

PATIENT ID: 2018056337

AGE: 67 y

ORDER: 3387424,3387425

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

FINDINGS:

Compared to the previous non-contrast CT study dated 21/12/2024, current one shows:

1-CHEST

- **Complete resolution** of the previously seen right breast lower inner quadrant irregular soft tissue lesion, was marked by a metallic clip (was measuring about 1.9 x 1.6 cm along its maximum axial dimensions). Still seen minimal regional skin thickening.
- **Mild progression in size** of the previously seen mildly enlarged right level I & II axillary lymph nodes, now the largest seen at level I, is measuring about 2.3 x 0.7 cm in axial diameter (compared to 1.6 x 0.7 cm previously).
- No appreciable interval changes regarding:
 1. prominent right internal mammary lymph node, measuring about 0.6 cm in diameter.
 2. Left breast few small nodular densities with small medial aspect benign looking calcified lesion, for sono-mammographic correlation.
 3. Few prominent bilateral supraclavicular lymph nodes, the largest measures about 0.6 cm in diameter.
 4. Left lower lung lobe few small calcified pulmonary nodules, likely granulomatous.
 5. Bilateral predominantly lower lobar mosaic attenuation with areas of air trapping intermingling with areas of parenchymal veiling.
 6. Middle lung lobe and lingular parenchymal bands.
- Otherwise, no sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Small sliding hiatus hernia.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-



- Still seen bilateral renal small stones and gravels, the largest seen at the left lower calyx, measuring about 0.2 cm in diameter.
- Normal appearance of the pancreas, spleen, kidneys (apart from few left renal simple cortical cysts) and adrenals.
- Aortoiliac atheromatous calcification.
- Underfilling of the urinary bladder showing mild circumferential mural thickening with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Small fat containing umbilical hernia.
- Non visualized uterus possibly surgically removed.
- Still seen right deep pelvic small cyst, measuring about 1.4 x 1 cm along its maximum axial dimensions, possibly adnexal in origin.
- No abdominal or pelvic masses or collections.

• **The revised bone window revealed:**

1. No newly developed sclerotic or lytic osseous lesions.
2. D1 small lucent area.
3. Dorsolumbar spondylosis.
4. Osteopenic texture of the examined bones.
5. Grade I-II lytic spondylolisthesis of L5.

Opinion:

Follow up study for a patient with right breast carcinoma showing:

- Complete resolution of the right breast lower inner quadrant spiculated neoplastic soft tissue lesion.
- Mild size progression of the mildly enlarged right axillary lymph nodes, possibly metastatic.
- Still no definite pulmonary, hepatic or newly developed metastatic deposits.

Reported By,

Dr. Sally Mahmoud, MSc

Dr. Marwa Ahmed, MD

21-May-2025 12:52 PM

PATIENT: NADYA IBRAHIM SULTAN MOHAMMED, 2018056337, Female, 67 y

Report version 6, 21-May-2025 12:52 PM

Page 2 of 2



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16602

مركز رعاية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
1. الهادي الأول - الشيخ زايد أمام كلية الهندسة

Patient's Name: Nadia Ibrahim Sultan Mohammed
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITHOUT CONTRAST ADMINISTRATION REVEALED:-

- The skin overlying the right breast is thickened and measures about 15mm. Non—homogenous density pattern of the right breast.
- Normal left breast.
- **Benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 15 mm in diameter.**
- Fine reticulations are seen in the middle lobe and in the anterior segment of right upper pulmonary lobe.
- Linear atelectatic bands are seen in the middle lobe,in the lingula,and in the lower pulmonary lobes.
- No sizable pulmonary nodules ,other patchy areas of consolidations or foci of calcifications noted.
- No sizable mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- **Border line size of the heart with atherosclerosis of its big vessels.**
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes and mild spondylolisthesis of L5 over S1 vertebral body.**
- Average sized liver,measuring about 15 cm, showing a homogenous parenchymal attenuation pattern.
- No sizable hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 8.9 cm. No focal lesions seen inside.
- Relatively thickened both adrenal glands.
- Normal CT appearance of the pancreas, both kidney's(apart from a left renal cyst,measuring about 20x20mm in dimensions and bilateral renal gravel), aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRES

IMPRESSIONS:

- The skin overlying the right breast is thickened with non—homogenous density pattern of the right breast. For clinical check up.
- Fine reticulations in the middle lobe and in the anterior segment of right upper pulmonary lobe with linear atelectatic bands in the middle lobe, lingula,and in both lower pulmonary lobes,likely to be a post-therapeutic sequelae.
- Relatively thickened both adrenal glands,likely to be a hyperplasia.
- Bilateral renal gravel.

Prof/Hanan El-Ahwal.

Patient's Name:

Nadia Ibrahim Sultan Mohammed

Date:

2026-08-25

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Dr. Hanan Alahwel

Dr. Hanan Alahwel

Patient's Name: Nadia Ibrahim Sultan Mohammed
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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- Bilateral renal gravel.

Prof/Hanan El-Ahwal.

Patient's Name:

Nadia Ibrahim Sultan Mohammed

Date:

2026-08-25

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Dr. Hanan Alahwel

Dr. Hanan Alahwel

Medical No. :

2614

Patient's Name:

Nadia Ibrahim Sultan Mohammed

Date:

2026-09-23

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Multi-Slice Post Contrast Axial CT Scan of the Neck, Chest and Abdomen with Coronal and 3D Reconstructions Revealed:

Neck Findings

- Normal CT scan of the nasopharynx, oropharynx and parapharyngeal spaces. No abnormal masses.
- Normal CT scan of the laryngeal structures. No supra-glottic, glottic or infra glottic masses.
- Normal appearance of the superficial and deep lobes of both parotid glands. No masses.
- Normal appearance of both submandibular glands. No masses.
- Normal both thyroid lobes, no nodules seen.
- No pathological masses could be noted in the neck.
- Few enlarged upper and lower deep cervical reactive looking LNs.
- Normal neurovascular bundles.

Chest findings

- Left basal reticulo-nodular density ...Likely inflammatory for clinical correlation and follow up.
- Bilateral basal atelectatic bands.
- No detected suspicious pulmonary nodules or masses.
- Normal central trachea with no lesions inside. Normal main bronchi.
- Normal both hilar region. No hilar masses or lymphadenopathy.
- Clear pleural surfaces with no evidence of pleural effusion.
- Mild enlarged cardiac size, no pericardial effusion.

Abdomen and pelvis findings

- Mild enlarged fatty liver. No abnormal attenuation values or enhancing lesions.
- No dilated intra-hepatic biliary radicles.
- Normal caliber patent Portal Vein
- Normal gallbladder. No hyperdense gall stones. Normal extrahepatic bile ducts.
- Normal size, shape and parenchymal density of the pancreas. No enhancing lesions.
- Normal size, shape and density of the spleen. No focal lesions.
- Normal size, and contrast excretion by both kidneys. Normal parenchymal thickness. No renal, masses stones or back pressure changes.
- Normal both supra-renal glands, no masses.
- No ascites
- Normal aorta, I.V.C.
- Normal filling of the urinary bladder with smooth inner surface and uniform wall thickness.
- No pelvic masses or collections. No abdominal or pelvic lymphadenopathy.
- Degenerative changes of the scanned bones, No aggressive bony lesion
- First degree spondylolisthesis of L5 over S1 vertebrae with bilateral pars defects.

Conclusion

- No detected suspicious pulmonary or hepatic deposits.

Ibrahim

HFL

PAOS. → 7x6cm

is there any HFL?

Yes

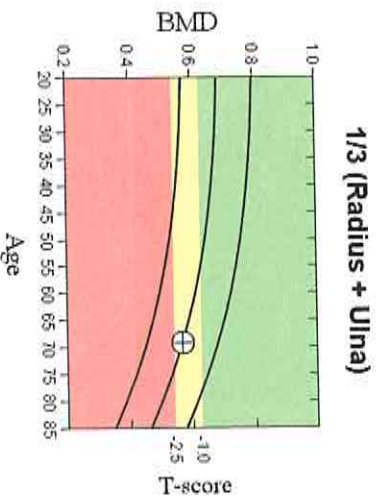
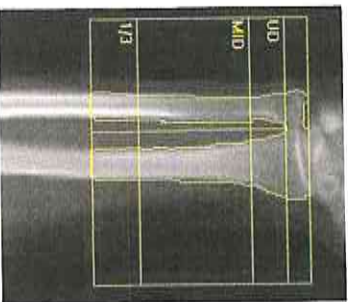
Name: NADIA IBRAHIM SOLTAN
Patient ID: 200920261
DOB: 02 July 1957

Sex: Female
Ethnicity: Hispanic

Height: 159.0 cm
Weight: 70.0 kg
Age: 69

Referring Physician:

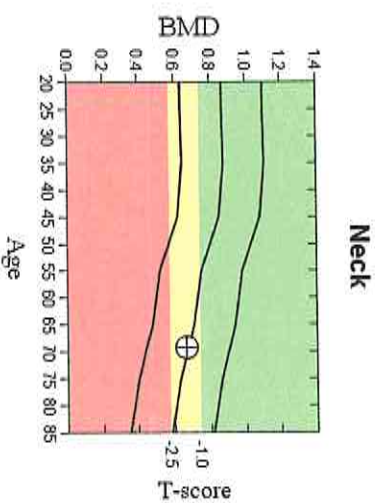
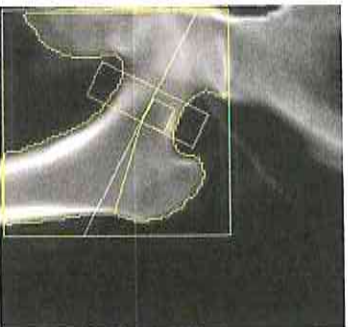
Scan Date: 20 September 2026 - A09202605



Radius + Ulna	BMD (g/cm²)	T - score	Z - score
1/3	0.569	-2.0	0.1

WHO Classification: Osteopenia
Fracture Risk: Increased

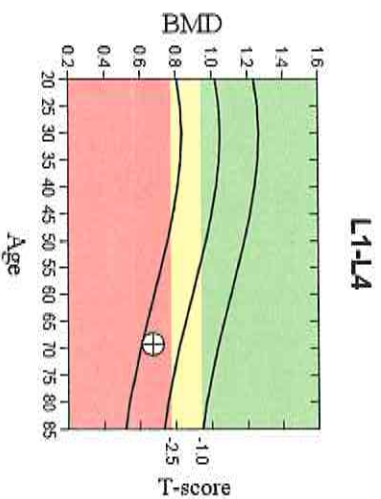
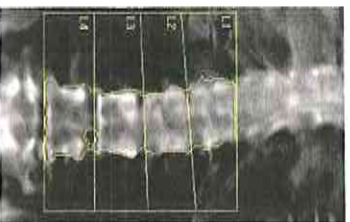
Image not for diagnostic use
228 x 94
DAP: 1.5 cGy*cm²



Region	BMD (g/cm²)	T - score	Z - score
Neck	0.664	-1.7	-0.1
Total	0.825	-1.0	0.4

WHO Classification: Osteopenia

Image not for diagnostic use
89 x 90
NECK: 47 x 15
DAP: 1.5 cGy*cm²



Region	BMD (g/cm²)	T - score	Z - score
L1-L4	0.670	-3.4	-1.4
L2-L4	0.665	-3.8	-1.6

WHO Classification: Osteoporosis
Fracture Risk: High

Image not for diagnostic use
116 x 113
DAP: 2.2 cGy*cm²

T-score vs. White Female. Source:2012 BMDCS/Hologic
Z-score vs. Hispanic Female. Source:Hologic

T-score vs. White Female. Source:2012 BMDCS/NHANES
White Female. Z-score vs. Hispanic Female.
Source:NHANES

Scan Date: 20 September 2026 - A09202604

10-year Fracture Risk
FRAX not reported because:
Some T-score for Spine Total or Hip Total or Femoral Neck at or below -2.5

Scan Date: 20 September 2026 - A09202603

Patient's Name: Nadia Ibrahim Sultan Mohammed
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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Prof/Hanan El-Ahwal.

Patient's Name:

Nadia Ibrahim Sultan Mohammed

Date:

2026-08-25

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Dr. Hanan Alahwel

Dr. Hanan Alahwel

Medical No. :

2614

Patient's Name:

Nadia Ibrahim Sultan Mohammed

Date:

2026-09-23

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



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Neck Findings

- Normal CT scan of the nasopharynx, oropharynx and parapharyngeal spaces. No abnormal masses.
- Normal CT scan of the laryngeal structures. No supra-glottic, glottic or infra glottic masses.
- Normal appearance of the superficial and deep lobes of both parotid glands. No masses.
- Normal appearance of both submandibular glands. No masses.
- Normal both thyroid lobes, no nodules seen.
- No pathological masses could be noted in the neck.
- Few enlarged upper and lower deep cervical reactive looking LNs.
- Normal neurovascular bundles.

Chest findings

- Left basal reticulo-nodular density ...Likely inflammatory for clinical correlation and follow up.
- Bilateral basal atelectatic bands.
- No detected suspicious pulmonary nodules or masses.
- Normal central trachea with no lesions inside. Normal main bronchi.
- Normal both hilar region. No hilar masses or lymphadenopathy.
- Clear pleural surfaces with no evidence of pleural effusion.
- Mild enlarged cardiac size, no pericardial effusion.

Abdomen and pelvis findings

- Mild enlarged fatty liver. No abnormal attenuation values or enhancing lesions.
- No dilated intra-hepatic biliary radicles.
- Normal caliber patent Portal Vein
- Normal gallbladder. No hyperdense gall stones. Normal extrahepatic bile ducts.
- Normal size, shape and parenchymal density of the pancreas. No enhancing lesions.
- Normal size, shape and density of the spleen. No focal lesions.
- Normal size, and contrast excretion by both kidneys. Normal parenchymal thickness. No renal, masses stones or back pressure changes.
- Normal both supra-renal glands, no masses.
- No ascites
- Normal aorta, I.V.C.
- Normal filling of the urinary bladder with smooth inner surface and uniform wall thickness.
- No pelvic masses or collections. No abdominal or pelvic lymphadenopathy.
- Degenerative changes of the scanned bones, No aggressive bony lesion
- First degree spondylolisthesis of L5 over S1 vertebrae with bilateral pars defects.

Conclusion

- No detected suspicious pulmonary or hepatic deposits.

Ibrahim

HFL

PAOS → 7x6cm

is there any HFL?

2x2.5

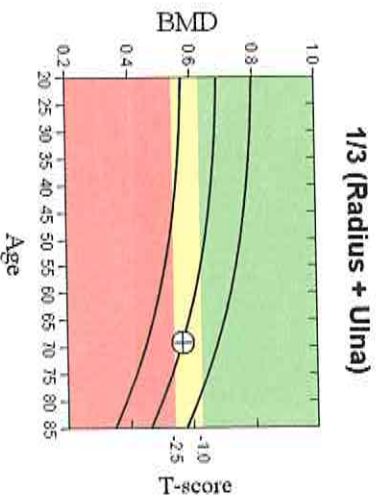
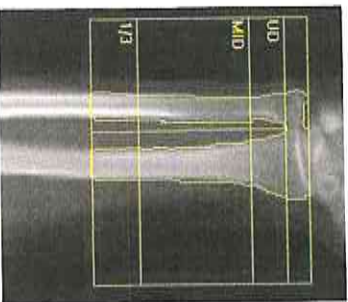
Name: NADIA IBRAHIM SOLTAN
Patient ID: 200920261
DOB: 02 July 1957

Sex: Female
Ethnicity: Hispanic

Height: 159.0 cm
Weight: 70.0 kg
Age: 69

Referring Physician:

Scan Date: 20 September 2026 - A09202605



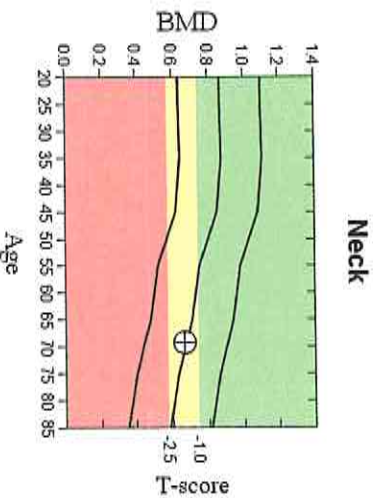
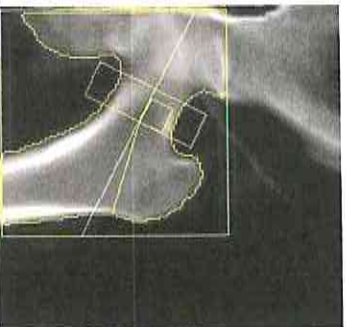
Radius + Ulna	BMD (g/cm²)	T - score	Z - score
1/3	0.569	-2.0	0.1

WHO Classification: Osteopenia
Fracture Risk: Increased

Image not for diagnostic use
228 x 94
DAP: 1.5 cGy*cm²

T-score vs. White Female. Source: Hologic
White Female. Source: Hologic

Scan Date: 20 September 2026 - A09202604



Region	BMD (g/cm²)	T - score	Z - score
Neck	0.664	-1.7	-0.1
Total	0.825	-1.0	0.4

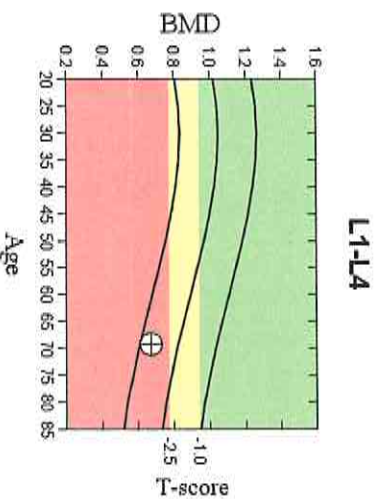
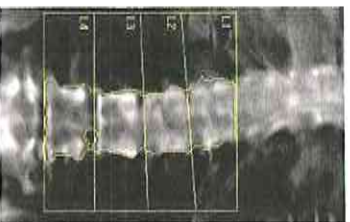
WHO Classification: Osteopenia

Image not for diagnostic use
89 x 90
NECK: 47 x 15
DAP: 1.5 cGy*cm²

T-score vs. White Female. Source: 2012 BMDCS/NHANES
White Female. Z-score vs. Hispanic Female.
Source: NHANES

10-year Fracture Risk
FRAX not reported because:
Some T-score for Spine Total or Hip Total or Femoral Neck at or below -2.5

Scan Date: 20 September 2026 - A09202603



Region	BMD (g/cm²)	T - score	Z - score
L1-L4	0.670	-3.4	-1.4
L2-L4	0.665	-3.8	-1.6

WHO Classification: Osteoporosis
Fracture Risk: High

Image not for diagnostic use
116 x 113
DAP: 2.2 cGy*cm²

T-score vs. White Female. Source: 2012 BMDCS/Hologic
Z-score vs. Hispanic Female. Source: Hologic

Patient's Name: Nadia Ibrahim Sultan Mohammed
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITHOUT CONTRAST ADMINISTRATION REVEALED:-

- The skin overlying the right breast is thickened and measures about 15mm. Non—homogenous density pattern of the right breast.
- Normal left breast.
- **Benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 15 mm in diameter.**
- Fine reticulations are seen in the middle lobe and in the anterior segment of right upper pulmonary lobe.
- Linear atelectatic bands are seen in the middle lobe,in the lingula,and in the lower pulmonary lobes.
- No sizable pulmonary nodules ,other patchy areas of consolidations or foci of calcifications noted.
- No sizable mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- **Border line size of the heart with atherosclerosis of its big vessels.**
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes and mild spondylolisthesis of L5 over S1 vertebral body.**
- Average sized liver,measuring about 15 cm, showing a homogenous parenchymal attenuation pattern.
- No sizable hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 8.9 cm. No focal lesions seen inside.
- Relatively thickened both adrenal glands.
- Normal CT appearance of the pancreas, both kidney's(apart from a left renal cyst,measuring about 20x20mm in dimensions and bilateral renal gravel), aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRES

IMPRESSIONS:

- The skin overlying the right breast is thickened with non—homogenous density pattern of the right breast. For clinical check up.
- Fine reticulations in the middle lobe and in the anterior segment of right upper pulmonary lobe with linear atelectatic bands in the middle lobe, lingula,and in both lower pulmonary lobes,likely to be a post-therapeutic sequelae.
- Relatively thickened both adrenal glands,likely to be a hyperplasia.
- Bilateral renal gravel.

Prof/Hanan El-Ahwal.

Patient's Name:

Nadia Ibrahim Sultan Mohammed

Date:

2026-08-25

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Dr. Hanan Alahwel

Dr. Hanan Alahwel